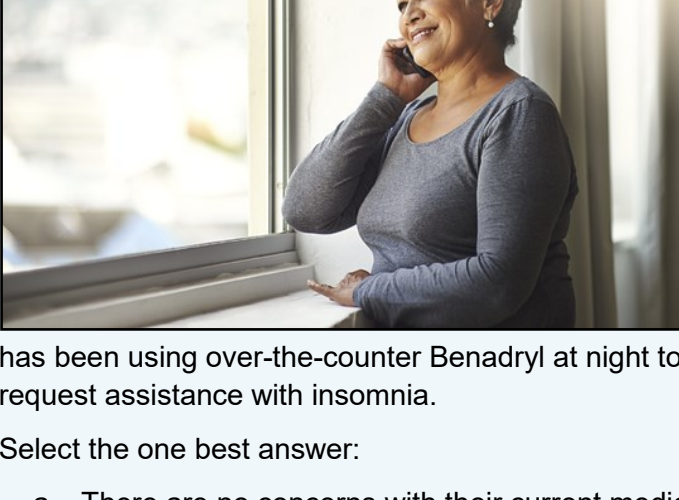


Medications to Avoid in Older Adults During Virtual Care

Did You Know?



A 76-year-old Female requests a telephone phone visit for “sleep problems.” A chart review shows they have hypertension, hyperlipidemia, DM 2 with CKD stage 4, hypothyroidism, osteoarthritis, mild cognitive impairment, history of falls, and non-melanoma skin cancer. Their active medications include amlodipine 5 mg daily, lisinopril 40 mg daily, metformin 500 mg twice daily, glipizide 5 mg twice daily, rosuvastatin 10 mg daily, levothyroxine 137 mcg daily, and meloxicam 7.5 mg daily.

Screening does not reveal any sleep hygiene concerns. Further questioning reveals the patient has been using over-the-counter Benadryl at night to help them sleep for the past few months. They request assistance with insomnia.

Select the one best answer:

- a. There are no concerns with their current medications. Continue all the medications and start melatonin 10 mg nightly for sleep.
- b. Due to the patient’s CKD, you recommend stopping meloxicam and all other non-steroidal, anti-inflammatory medications. Recommend they use acetaminophen as needed for arthritis pain. Add Ambien 2 mg nightly for sleep.
- c. There are concerns with both meloxicam and Benadryl use in this patient. Advise they stop both medications and explain rationale(s). Recommend acetaminophen for osteoarthritis pain, then offer Lunesta 1 mg nightly for insomnia. Arrange PCP follow-up for further prescriptions.
- d. There are concerns with both meloxicam and Benadryl. Advise they stop both medications and explain rationale(s). Recommend acetaminophen for osteoarthritis pain. Refer patient to Health Education for insomnia class. Offer a trial of melatonin titration starting with 3mg nightly for insomnia.
- e. Due to the patient’s history of falls, you advise they stop over-the-counter Benadryl. You recommend referrals to Health Education for insomnia class, sleep medicine for OSA testing, and that the patient come in for an examination prior to ordering any new medications.

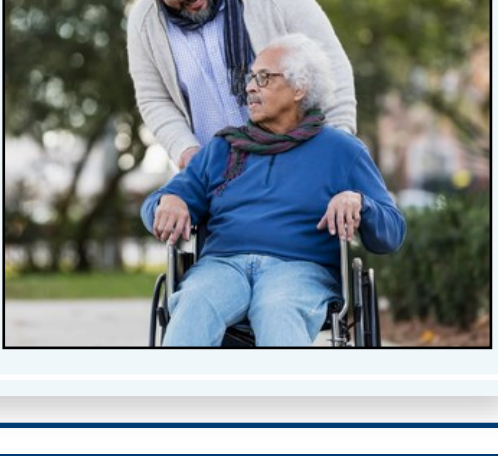
The answer is d.

Did You Know?

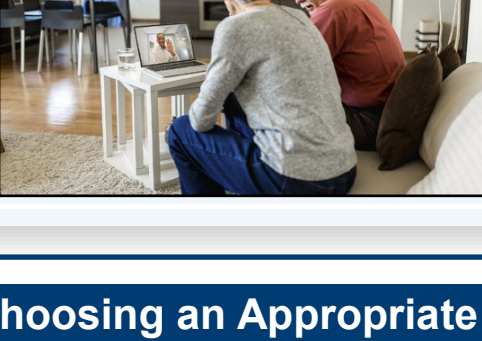
Older adults (≥65 years old) take prescribed medications [more frequently than any other age group](#) in the United States.

Why does this matter?

- As we age, our bodies take more time to break down medications, causing them to stay in our systems longer.
- Medications that treat one condition may worsen another.
- The possibility of serious drug-drug interactions increases the longer a medication stays in our system, and the more medications we take.
- Many older adults also take over-the-counter (OTC) medications, vitamins, or supplements, and **may not always disclose these during virtual care visits.**



Virtual Practice Pearls: Prescribing Medication



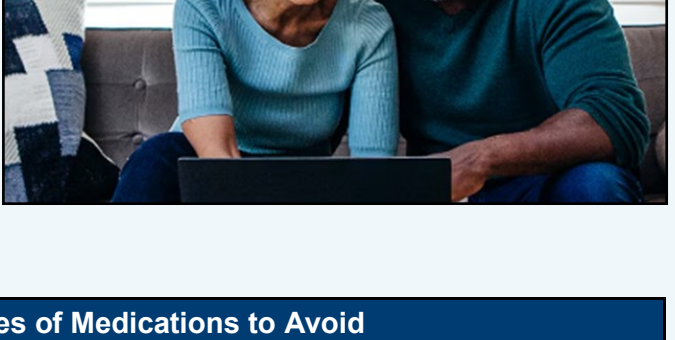
Patients ≥65 years old have a higher risk of overmedication and unwanted drug interactions.

It is important to be mindful of medication prescribing, usage, and interactions in our older adult population who are increasingly utilizing our virtual workstream services, like Get Care Now. This is key to our Regional Virtual Medical Center’s mission of providing safe, high-value virtual medical care.

Choosing an Appropriate Medication

The AGS Beers Criteria

- Developed by the late Dr. Mark Beers and colleagues in 1991, now maintained and updated by the American Geriatric Society (AGS), the Beers Criteria are a set of guidelines and recommendations aimed at improving the appropriate use of medications in older adults.
- Lists >50 potentially inappropriate medications (PIMs) that pose risks for adults ≥65 years.
- Medication harms can take a toll on everyone involved and can have devastating consequences.
- [The complete Beers 2023 medication guide.](#)



Medications to Avoid in Adults ≥65 Years

Beers Examples of Categories of Medications to Avoid	
Anticholinergics	Non-steroidal anti-inflammatory drugs
Antidepressants (with strong anticholinergic activity)	Opioids
Antipsychotics	Proton-pump inhibitors (avoid use >8 weeks unless high-risk)
Benzodiazepines	Skeletal muscle relaxants

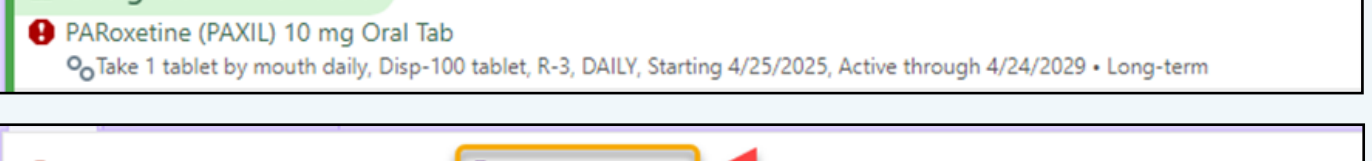
Specific Medications You May Reconsider Prescribing Virtually to Older Patient

Medication	Risk	Recommendation
Diphenhydramine (Benadryl)	Confusion, dry mouth, constipation, falls, other anticholinergic toxicity	Avoid ≥65 years
Hydroxyzine (Atarax)	Confusion, dry mouth, constipation, falls, other anticholinergic toxicity	Avoid ≥65 years
Nitrofurantoin (Macrobid)	Pulmonary toxicity, hepatotoxicity. Peripheral neuropathy in long-term use.	Avoid if CrCl<30mL/min Avoid for long-term suppression
Nortriptyline (Pamelor)	Sedation, orthostatic hypotension, highly anticholinergic	Avoid ≥65 years
Omeprazole	C. Difficile infection, pneumonia, GI malignancies, bone loss and fractures	Avoid scheduled use >8 weeks unless for high-risk patients (e.g., Barrett’s esophagitis)
Paroxetine (Paxil)	Sedation, orthostatic hypotension, highly anticholinergic	Avoid ≥65 years
Scopolamine	Highly anticholinergic	Avoid ≥65 years

HealthConnect Can Aid in Choosing Medications

Checking for Drug-Drug Interactions

- First, review active medications to check for drug-drug interactions.
- Enter your new medication order.
- Select **New Interactions** prior to clicking **Sign Orders**.



Checking for Drug-Disease Interactions

You can check for drug-disease interactions within the **Problems List** navigator. In the upper-right corner, select the **View Drug-Disease Interactions** button if you are uncertain of potential contraindications or interactions.





Drug-Disease: diphenhydrAMINE and CAD WO ANGINA, W HX OF MI
Moderate Warning for coronary artery disease
Filtered by system settings
[Details](#)

You can view the **Details** section to get more information on the specific drug-disease warning.

Current Warnings (46 filtered)



Drug-Disease: Topiramate and MAJOR DEPRESSIVE DISORDER, RECURRENT EPISODE, IN FULL REMISSION; MAJOR DEPRESSIVE DISORDER, SINGLE EPISODE; MAJOR DEPRESSIVE DISORDER, RECURRENT EPISODE
Severe Warning for depression
Filtered by system settings
[Details](#)

Topiramate (TOPAMAX) 25 mg Oral Tab
Prescription. Active. Multiple dosages.

Discontinue

Heed HealthConnect’s Safety Alerts

Please follow the medication safety alert “pop-ups” in HealthConnect.

Safety Alert (1)

SAFETY ALERT!

Select antidepressants are **HIGH RISK** medications for patients > 64 years of age due to increased risk of falls and cognitive impairment.
ACTION: Click ‘Accept’ to remove entered order and select alternate order(s) below.
[Falls and Cognitive Impairment - Journal Reference](#)

Remove the following orders?

RemoveKeep

PARoxetine (PAXIL) 10 mg Oral Tab
Take 1 tablet by mouth daily, Disp-100 tablet, R-3, DAILY, Starting 4/25/2025, Active through 4/2...

Apply the following?

Order	Do Not Order	Neuropathy: OTC, lidocaine 5 % top crm (LM.X.5), bid prn pain
Order	Do Not Order	Neuropathy: OTC, capsaicin 0.025% top crm (ZOSTRIX), tid prn
Order	Do Not Order	Neuropathy/GAD/MDD (Avoid for falls/fractures and if CrCl <30 mL/min): DULoxetine 20 mg cap (CYMBALTA), 1 daily
Order	Do Not Order	GAD/MDD/IBS (Avoid w/ falls/fractures): sertraline 25 mg tab (ZOLOFT), 1 daily
Order	Do Not Order	GAD only: busPIRone 5 mg tab (BUSPAR), 1 bid
Order	Do Not Order	Insomnia (w/ depression): mirtazapine 15 mg tab (REMERON), 0.5 at bedtime
Order	Do Not Order	Insomnia: OTC, melatonin 3 mg tab, 1 po 30 mins before bedtime prn
Order	Do Not Order	Insomnia: traZODone 50 mg tab (DESYREL), STARTING DOSE, 0.5 at bedtime prn

Safety Alert (1)

SAFETY ALERT!

Select Antitussive/Anticholinergic combination drugs are **HIGH RISK** medications for patients > 64 years of age due to side effects associated with sedation, confusion and increased risk of falls.
ACTION: Click ‘Accept’ to remove entered order and select alternate order(s) below.
[Falls and Cognitive Impairment - Journal Reference](#)

Remove the following orders?

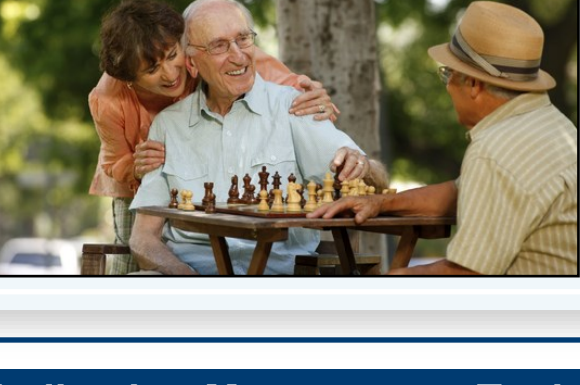
RemoveKeep

Scopolamine Base (TRANSDERM-SCOP) 1 mg over 3 days TD 3 Days Patch
Apply 1 Patch to skin every 72 hours as needed for motion sickness or nausea . Apply behind t...

Apply the following?

Order	Do Not Order	Cough: OTC, ROBITUSSIN DM (10-100 mg/5 mL) syrup, 1 tsp q4h prn
Order	Do Not Order	Cough: OTC, guaIFENesin 100 mg/5 mL liq (ROBITUSSIN), 1 tsp q4h prn
Order	Do Not Order	Cough: OTC, guaIFENesin 100 mg/5 mL (sugar free) liq (DIABETIC TUSSIN EX), 1 tsp q4h prn
Order	Do Not Order	Nausea: ondansetron 4 mg tab (ZOFTRAN), 1 q12h prn
Order	Do Not Order	Nausea: ondansetron 4 mg dis tab (ZOFTRAN ODT), 1 q12h prn
Order	Do Not Order	Nausea: ondansetron 8 mg tab (ZOFTRAN), 1 q12h prn
Order	Do Not Order	Nausea: ondansetron 8 mg dis tab (ZOFTRAN ODT), 1 q12h prn
Order	Do Not Order	Motion sickness: OTC, SEA-BAND ADULT, use as directed

May is Older Americans Month

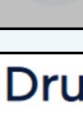


Since 1963, the United States has honored older Americans during May. The theme of 2025 is “Flip the Script on Aging,” focusing on transforming how society perceives, talks about, and approaches aging.

In raising awareness of potentially inappropriate medications (PIMs) on virtual encounters, Permanente providers promote age-friendly care, not to mention general public health and safety.

Medication Management Tools

KP Patient Medication Management Tools



Managing your medicine
Kaiser Permanente provides tips on how to manage your medications as a senior.

Drugs to approach with care

The medicines below should be avoided or used cautiously because older adults are sensitive to their side effects. If you see a drug that you are taking on the list below*, check with your physician. Do not stop taking any medication without first talking to your doctor. You can find additional information and a full list of medications to avoid or use with caution through the [Health in Aging](#) website.

MEDICATION	REASON
Non-steroidal anti-inflammatory drugs (NSAIDs) - Long-acting NSAIDs like indomethacin (Indocin) and piroxicam (Feldene) - Shorter-acting NSAIDs like ibuprofen (Advil, Motrin) and salsalate (Disalcid) are better choices	NSAIDs can increase the risk of indigestion, ulcers, and bleeding in your stomach or colon. They can also increase blood pressure, affect your kidneys, and make heart failure worse.
Digoxin (Lanoxin) in doses greater than 0.125 mg.	It can be toxic in older adults and people whose kidneys do not work well.
Certain diabetes drugs - Glyburide (Diabeta, Micronase) - Chlorpropamide (Diabinese) - Glimepiride (Amaryl)	These can cause severe low blood sugar.
Muscle relaxants - Cyclobenzaprine (Flexeril) - Methocarbamol (Robaxin) - Carisoprodol (Soma), and similar medications.	They can leave you feeling groggy and confused, increase your risk of falls, and cause constipation, dry mouth, and problems urinating. Plus, there is little evidence that they work well.

Other Patient Medication Management Resources

[Tip Sheet: Avoiding Overmedication and Harmful Drug Reactions.](#)

References

American Geriatrics Society. (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. *Journal of the American Geriatrics Society*. <https://sbgg.org.br/wp-content/uploads/2023/05/1-American-Geriatrics-Society-2023.pdf>

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